

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033
Email: centrallab@luciddiagnostics.comClient : HCL AVITAS PVT LTD
Patient Name : **Mr.ANIL KUMAR REDDY**Age / Gender : **38 Y(s) / Male**

Phone : 8008931117

Ref.Dr. : SELF

Req.No : **JBH092500684** - 2500796989Report Status : **Final**

Sample Type : Plasma - PP

Registered On : 20-09-2025 09:17

Collected On : 20-09-2025 12:31

Received On : 20-09-2025 13:18

Reported On : 20-09-2025 14:42

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DEPARTMENT OF CLINICAL BIOCHEMISTRY

Glucose, Post Lunch (PLBS)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Post Lunch Plasma Glucose	: 109	mg/dL	100-140

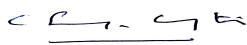
Method : Hexokinase

Remarks:

Postprandial hypoglycemia may be observed in persons taking certain diabetes medications, antibiotics and alcoholic beverages. Additionally intestinal disorders, endocrine disorders, insulin surge, nature of diet, reduced food intake than usual and stress are among several other factors. Incretin effect: Incretins released by the neuroendocrine cells of the GI tract can cause an insulin surge that can lead to lowering of the blood glucose level in the post prandial state, some times to levels below the fasting level. Please correlate clinically.

Reference:American Diabetes association guidelines 2021.

Note :- Suggested Clinical Correlation *

****End Of Report******Dr.Vijay Babu B,MBBS.MD**
Consultant Biochemistry**Dr.Sowmya Gayatri .C**
M.B.B.S MD
Consultant Biochemistry**Dr.S.A.Nabi Ph.D**
Sr.Biochemist

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Phone : 8008931117

Ref.Dr. : SELF

Req.No : **JBH092500684** - 2500796541Report Status : **Preliminary**

Sample Type : Serum

Registered On : 20-09-2025 09:17

Collected On : 20-09-2025 11:12

Received On : 20-09-2025 11:19

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DEPARTMENT OF CLINICAL BIOCHEMISTRY

Blood Urea Nitrogen (BUN)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Blood Urea	: 23.29	mg/dL	17-43

Method : GLDH Kinetic

Interpretation: -**Increased In**

Impaired kidney function, congestive heart failure, salt and water depletion due to vomiting, diarrhea urinary tract obstruction, hemorrhage into GI tract.

Decreased In

Malnutrition, pregnancy, infancy, acromegaly, low protein- High carbohydrate diet, impaired absorption, some patients of nephrotic syndrome.

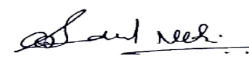
Blood Urea Nitrogen : 10.9 mg/dL 7.0-18.0

Method : Calculated

Reference

In blood, Urea is usually reported as BUN and expressed in mg/dl. BUN mass units can be converted to urea mass units by multiplying by 2.14. Beckman kit literature.

Note :- Suggested Clinical Correlation *

****End Of Report******Dr.Vijay Babu B,MBBS.MD**
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DEPARTMENT OF CLINICAL BIOCHEMISTRY

Calcium, Serum

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Calcium, Serum	: 9.7	mg/dL	8.8-10.6

Method : Arsenazo III

Interpretation**Increased :**

Increased levels may be seen in hyperparathyroidism, vitamin D intoxication, milk-alkali syndrome, thiazide diuretic use, and granulomatous diseases

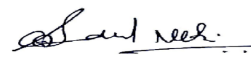
Decreased :

Hypoparathyroidism, chronic renal disease with uremia and phosphate retention, malabsorption of calcium and vitamin D, and insufficient calcium, phosphorus, and vitamin D intake.

Reference:

Beckman kit Insert.

Note :- Suggested Clinical Correlation *

****End Of Report******Dr.Vijay Babu B,MBBS.MD**
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DEPARTMENT OF CLINICAL BIOCHEMISTRY

Creatinine, Serum

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Creatinine,Serum	: 0.99	mg/dL	0.67-1.17

Method : Modified Jaffe Kinetic

Reference :Tietz fundamentals of clinical chemistry, 7 th edition.

Interpretation:

Increased In

- * Diet: ingestion of creatinine (roast meat)
- * Muscle disease: gigantism, acromegaly
- * Prerenal azotemia
- * Impaired kidney function
- * An increase in serum creatinine creatinine occurs in 10.20% of patients taking aminoglycosides and $\leq 20\%$ of patients taking penicillins (especially methicillin)

Decreased In

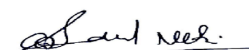
- * Creatinine secretion is inhibited by certain drugs (e.g., cimetidine, trimethoprim).
- * Proxy for reduced skeletal muscle mass.

Note :- Suggested Clinical Correlation *

****End Of Report****

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Sample Type : Serum

Registered On : 20-09-2025 09:17

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Reported On : 20-09-2025 13:53

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DEPARTMENT OF SEROLOGY

High Sensitivity - CRP (HS-CRP)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
hs-CRP	: 0.39	mg/dL	Low risk : less than 0.1 Average risk : 0.1 to 0.3 High risk : above 0.3

Method : Immunoturbidimetry

Reference :

The American Heart Association and U.S. Centers for Disease Control and Prevention.

Note:

In addition to hs-CRP, other additional risk factors to be considered for cardiovascular diseases (CVD)

Note :- Suggested Clinical Correlation *

End Of Report

Dr.Archana G J MBBS MD
Consultant Microbiologist
Dr.Sara Shireen MBBS MD
Consultant Microbiologist

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DEPARTMENT OF CLINICAL BIOCHEMISTRY

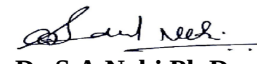
Phosphorous, Serum

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Phosphorous, Serum	: 2.4	mg/dL	2.5 -4.5

Method : Phosphomolybdate

Reference:Beckman Kit Insert.

Note :- Suggested Clinical Correlation *

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DEPARTMENT OF CLINICAL BIOCHEMISTRY

Transferrin Saturation

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Iron <i>Method : TPTZ</i>	: 134.20	µg/dL	70-180
Total Iron Binding Capacity (TIBC) <i>Method : Nitroso-PSAP</i>	: 486.05	µg/dL	255-450
Unsaturated Iron Binding Capacity (UIBC) <i>Method : Nitroso-PSAP</i>	: 351.85	µg/dL	155-300
Transferrin Saturation <i>Method : Calculation</i>	: 27.61	%	20-50

Reference: Beckman kit Insert.

Note :- Suggested Clinical Correlation *

****End Of Report****

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DEPARTMENT OF CLINICAL BIOCHEMISTRY

Uric Acid, Serum

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Uric Acid	: 5.49	mg/dL	3.5-7.2

Method : Uricase PAP(Phenyl AminoPhenazone)

Interpretation

Increased :

renal failure, gout, leukemia, polycythemia, and hemolytic anemia. decreased renal clearance due to drugs like thiazides

and furosemide, the use of drugs like barbiturates, some patients with alcoholism, high protein weight-reduction diet.

Decreased :

Use of uricosuric drugs like probenecid, Allopurinol, cortisone, estrogens, Fanconi syndrome, etc.

Note :- Suggested Clinical Correlation *

End Of Report

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DEPARTMENT OF CLINICAL BIOCHEMISTRY

Vitamin B12 (Cyanocobalamin),Serum

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Vitamin B12 (Cyanocobalamin), Serum	: 156	pg/mL	222-1439

Method : CLIA

Note : Results released pertain to the specimen submitted and are dependent on the quality of the sample received by the laboratory. Repeat the test in case of clinical discordance.

Interpretation:

1.Vitamin B12 deficiency occurs in megaloblastic anaemia,malabsorption,dietary deficiency(vegeterians),congenital disorders,pregnancy etc.

2.An associated increase in homocysteine is an independent marker for cardiovascular disease and deep vein thrombosis.

3.Holotranscobalamin is a more sensitive marker of Vitamin B12 deficiency.

4.Elevated Vitamin B12 is associated with use of oral contraceptives,multivitamins ,Chronic renal failure,liver disease,increased levels of transcobalamin,acute and chronic myelogeneous leukemia etc.

Note: disagreement of vitamin b12 values can be noticed between laboratories using assays of different manufacturers.

Reference:Beckman caliper study of pediatric Reference values.Beckman kit insert,Teitz Fundamentals of clinical chemistry.

Note :- Suggested Clinical Correlation *

****End Of Report****

[Signature]

Dr.Vijay Babu B,MBBS.MD
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Age / Gender : **38 Y(s) / Male**
Phone : 8008931117
Ref.Dr. : SELF
Req.No : **JBH092500684** - 2500796541
Report Status : **Preliminary**

Sample Type : Serum
Registered On : 20-09-2025 09:17
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Received On : 20-09-2025 11:19
Reported On : 20-09-2025 13:31

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DEPARTMENT OF CLINICAL BIOCHEMISTRY

Lipid Profile

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Total Cholesterol <i>Method : CHOD-POD</i>	: 226.0	mg/dL	Desirable: < 200 Borderline: 200 – 239 High: >= 240
HDL Cholesterol <i>Method : Enzymatic Immunoinhibition</i>	: 49.1	mg/dL	Major risk factor for heart disease: <40 Negative risk factor for heart disease: >60
LDL Cholesterol <i>Method : Calculation</i>	: 149.68	mg/dL	Optimal : < 100 Near Optimal : 100–129 Borderline High : 130-159 High : 160–189 Very High : > 190
VLDL <i>Method : Calculated</i>	: 27.22	mg/dL	6-38
Triglycerides <i>Method : Glycerol Phosphate Oxidase (GPO), Peroxidase (POD)</i>	: 136.08	mg/dL	Normal: < 150 Borderline High: 150 - 199 High: 200 - 499 Very High: >= 500
CHOL / HDL Ratio <i>Method : Calculation</i>	: 4.60		3.5-5.0
Non - HDL Cholesterol <i>Method : Calculated</i>	: 176.90	mg/dL	<130
LDL/HDL Ratio <i>Method : Calculated</i>	: 3.05	mg/dL	1.3-3.5

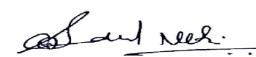
Reference:The National Cholesterol Education Program Adult Treatment Panel III (NCEP-ATP III) Guidelines.

Note :- Suggested Clinical Correlation *

****End Of Report****

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Consultant Biochemistry

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Dr.S.A.Nabi Ph.D
Sr.Biochemist

Regd. Office:

Print Date : 20-09-2025 14:39

Page 9 of 20

Plot # 18, Rao & Raju Colony, Road No.2, Banjara Hills, Hyderabad-500 034.
Ph: +91 40 4419 4444, Toll Free No. : 1800 123 4577

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DEPARTMENT OF CLINICAL BIOCHEMISTRY

Thyroid Profile (T3,T4,TSH)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Total T3 <i>Method : CLIA</i>	: 1.68	ng/mL	0.87-1.78
Total T4 <i>Method : CLIA</i>	: 10.60	µg/dL	4.82-15.65
TSH <i>Method : CLIA</i>	: 1.93	µIU/mL	Children: Birth-4d:1.0-39.0 2-20wk:1.7-9.1 21wk-20y:0.7-6.4 Adults: 21-54y:0.4-4.2 55-87y:0.5-8.9

Interpretation :

1.T3 &T4 values may be altered due to changes in serum proteins,pregnancy,drugs,nephrosis etc.In such cases Free T3 and Free T4 may give more appropriate thyroid status.T3 levels fluctuate rapidly to stress and non thyroid illness.

2.TSH values may be transiently altered in fever,severe infections,liver disease,renal and heart failure,severe burns,trauma and surgery.

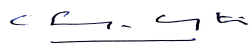
3.Drugs that decrease TSH values include L-DOPA,Glucocorticoids,Heparin.Drugs that increase TSH include-Iodine,Lithium,Amiodarone.

4.TSH exhibits diurnal rythm,peaks at 2.00- 4.00AM and has lowest level at 5.00-6.00PM

Reference: Beckman caliper study of pediatric Reference values.Beckman kit insert,Teitz Fundamentals of clinical chemistry.

Note :- Suggested Clinical Correlation *

****End Of Report****



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DEPARTMENT OF CLINICAL BIOCHEMISTRY

Liver Function Test (LFT)

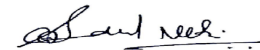
TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Total Bilirubin <i>Method : DPD</i>	: 0.79	mg/dL	0.3-1.2
Direct Bilirubin <i>Method : DPD</i>	: 0.15	mg/dL	<0.2
Indirect Bilirubin <i>Method : Calculated</i>	: 0.64	mg/dL	0.0-0.9
SGPT / ALT <i>Method : IFCC without Pyridoxal Phosphate</i>	: 17.93	U/L	0-50
AST/SGOT <i>Method : IFCC without Pyridoxal Phosphate</i>	: 22.55	U/L	0-50
Alkaline Phosphatase <i>Method : IFCC AMP-Buffer</i>	: 84.35	IU/L	30-120
Total Protein (TP) <i>Method : Biuret</i>	: 7.56	g/dL	6.6-8.3
Albumin <i>Method : Bromocresol Green(BCG)</i>	: 4.36	g/dL	3.5-5.2
Globulin <i>Method : Calculation</i>	: 3.2	g/dL	1.8-3.8
A/G Ratio <i>Method : Calculated</i>	: 1.36		0.9-1.8

Note :- Suggested Clinical Correlation *

****End Of Report****

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Age / Gender : **38 Y(s) / Male**

Phone : 8008931117

Ref.Dr. : SELF

Req.No : **JBH092500684** - 2500796540

Report Status : Preliminary

Sample Type : **WHOLE BLOOD EDTA** Scan to Validate

Registered On : 20-09-2025 09:17

Collected On : 20-09-2025 11:12

Received On : 20-09-2025 11:19

Reported On : 20-09-2025 12:25



DEPARTMENT OF CLINICAL BIOCHEMISTRY

Glycosylated Hb (HbA1C)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Glycosylated Hb (HbA1C)	: 5.9	%	Normal:<5.7 Prediabetes:5.7-6.4 Diabetes:>6.5
<i>Method : High-performance liquid chromatography Boronate affinity</i>			
Estimated Average Glucose	: 122.63	mg/dL	
<i>Method : Calculated</i>			

Use:

HbA1C reflects the mean blood glucose concentration over the previous 3-4 months.

Interpretation:

Criteria for diagnosis of Diabetes:

HbA1C $\geq$ 6.5% using method that is NGSP certified & standardised to DCCT Assay.

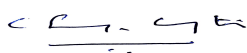
Note:

Low HbA1C values(<4%) in an individual are often associated with systemic inflammatory diseases, chronic anemia, chronic renal failure and liver diseases.

Reference: American Diabetes Association guidelines 2022.

Note :- Suggested Clinical Correlation *

****End Of Report****



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Req.No : **JBH092500684** - 2500796543
Report Status : **Final**

Sample Type : Urine
Registered On : 20-09-2025 09:17
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Scan to Validate



DEPARTMENT OF CLINICAL PATHOLOGY

Complete Urine Examination (CUE)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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PHYSICAL EXAMINATION:

Colour : Pale Yellow STRAW TO YELLOW
Appearance : Clear

CHEMICAL EXAMINATION

Reaction (pH) : Neutral 7.0 5.0-8.0

Method : Methyl Red & Bromothymol Blue

Specific Gravity : 1.005 1.000-1.030

Method : pKA Change

Protein / Albumin : Negative NEGATIVE

Method : Tetrabromophenol blue

Glucose / Sugar : Negative NEGATIVE

Method : Glucose oxidase/Peroxidase

Blood : Negative NEGATIVE

Method : Peroxidase

Ketones : Negative NEGATIVE

Method : Sodium Nitroprusside

Bilirubin : Negative NEGATIVE

Method : Dichloroanilinediazonium

Leucocytes : Negative NEGATIVE

Method : 3 hydroxy5 phenylpyrrole + diazonium

Nitrites : Negative NEGATIVE

Method : Diazonium + 1,2,3,4 tetrahydrobenzo (h) quinolin 3-ol

Urobilinogen : 0.2 mg/dL 0.2-1.0

Method : Dim ethyl aminobenzaldehyde

MICROSCOPIC EXAMINATION:

Pus Cells : 3-4 /HPF 0-5

Epithelial Cells : 1-2 /HPF 0-5

RBC : Absent /HPF Absent

Casts : Absent Absent

Crystals / Amorphous deposit : Absent Absent

Others : Nil Nil

Method: Reflectance Photometry, Automated Urine Analyzer/Microscopy.

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Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD
Patient Name : **Mr.ANIL KUMAR REDDY**
Age / Gender : **38 Y(s) / Male**
Phone : 8008931117
Ref.Dr. : SELF
Req.No : **JBH092500684** -
Report Status : **Final**

Sample Type : Urine
Registered On : 20-09-2025 09:17
Collected On : 20-09-2025 11:12
Received On : 20-09-2025 11:19
Reported On : 20-09-2025 13:23

Scan to Validate



Reference:

Siemens Multistix 10 SG Kit insert. Textbook of Medical Laboratory Technology B. Godkar, 3rd Edition.

Note :- Suggested Clinical Correlation *

****End Of Report****

Dr.A.Ramya Priyadarshini
Consultant Pathologist



Dr.K Kiran
Consultant Pathologist

Dr.Gaurav Rastogi
MD, Pathology
Senior Consultant

Dr.Sushmitha Reddy A
Consultant Pathologist

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD
Patient Name : **Mr.ANIL KUMAR REDDY**
Age / Gender : **38 Y(s) / Male**
Phone : 8008931117
Ref.Dr. : SELF
Req.No : **JBH092500684** - 2500796540
Report Status : **Final**

Sample Type : **WHOLE BLOOD EDTA** Scan to Validate
Registered On : 20-09-2025 09:17
Collected On : 20-09-2025 11:13
Received On : 20-09-2025 11:19
Reported On : 20-09-2025 13:53



DEPARTMENT OF HAEMATOLOGY

Complete Blood Picture (CBP)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Hemoglobin <i>Method : Non-Cyanide Photometric Measurement</i>	: 14.8	g/dL	13.0-17.0
HCT/Haematocrit <i>Method : Calculated</i>	: 44.8	%	40.0-50.0
RBC Count <i>Method : Electrical Impedance</i>	: 5.47	millions/cumm	4.5-5.5
MCV <i>Method : Calculated</i>	: 81.9	fL	83.0-101.0
MCH <i>Method : Calculated</i>	: 27.1	pg	27.0- 32.0
MCHC <i>Method : Calculated</i>	: 33.1	g/dL	31.5-34.5
RDW-CV <i>Method : Calculated</i>	: 15.6	%	11.6-14.0
RDW-SD <i>Method : Calculated</i>	: 45.1	fL	39-46
MPV <i>Method : Calculated</i>	: 8.6	fL	7.40-10.40
Platelet Count <i>Method : Electrical Impedance</i>	: 3.11	lakhs/cmm	1.50-4.10
Total WBC Count <i>Method : Electrical Impedance</i>	: 7800	Cells/cmm	4000-11000

Differential Count

(Method:VCS/Leishman Stain/Microscopy)

Neutrophils	: 48	%	40-80
Lymphocytes	: 44	%	20-40
Monocytes	: 06	%	2-10
Eosinophils	: 02	%	1-6
Basophils	: 00	%	0-1

PERIPHERAL BLOOD PICTURE

RBC	: Normocytic Normochromic
WBC	: Normal in morphology and distribution
Platelets	: Adequate

Print Date : 20-09-2025 14:39

Page 15 of 20

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Processing Location :

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Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD
Patient Name : **Mr.ANIL KUMAR REDDY**
Age / Gender : **38 Y(s) / Male**
Phone : 8008931117
Ref.Dr. : SELF
Req.No : **JBH092500684** -
Report Status : **Final**

Sample Type : WHOLE BLOOD EDTA
Registered On : 20-09-2025 09:17
Collected On : 20-09-2025 11:13
Received On : 20-09-2025 11:19
Reported On : 20-09-2025 13:53



Reference : Dacie and Lewis Practical Hematology, 12th Edition

Note : These results are generated by a fully automated hematology analyzer and the differential count is done on a peripheral smear.

Method:

Fully automated haematology analyzer (Beckman Coulter DXH900) (Photometric Measurement, Electrical Impedance, VCS Technology, Leishman's Stain and Microscopy)

Note :- Suggested Clinical Correlation *

****End Of Report****

Dr.Gaurav Rastogi MD, Pathology
Senior Consultant

Dr.Sushmitha Reddy A
Consultant Pathologist


Dr.K Kiran
Consultant Pathologist

Dr.A.Ramya Priyadarshini
Consultant Pathologist

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD
Patient Name : **Mr.ANIL KUMAR REDDY**
Age / Gender : **38 Y(s) / Male**
Phone : 8008931117
Ref.Dr. : SELF
Req.No : **JBH092500684** - 2500796540
Report Status : **Final**

Sample Type : **WHOLE BLOOD EDTA** Scan to Validate
Registered On : 20-09-2025 09:17
Collected On : 20-09-2025 11:12
Received On : 20-09-2025 11:19
Reported On : 20-09-2025 13:53



DEPARTMENT OF HAEMATOLOGY

Erythrocyte Sedimentation Rate (ESR)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
E.S.R 1st HOUR	: 20	mm/hr	Male: 0 – 10 mm/1st hr Female: 0 – 15 mm/1st hr

Method : Modified Westergren

Interpretation:

Erythrocyte sedimentation rate (ESR) is a useful but nonspecific marker of underlying inflammation. ESR is elevated in: Rheumatoid arthritis, chronic infection, collagen disease, polyclonal hyperglobulinemia and hyperfibrinogenemia Temporal arteritis, septic arthritis, pelvic inflammatory disease, and appendicitis, Osteomyelitis. Neoplastic disease- Myeloma or Macroglobulinemia, Prostate cancer, Hodgkin's disease, Renal cell carcinoma. Stroke, coronary artery disease Pregnancy - increase at the 10th to the 12th week, and returns to normal about 1 month postpartum. ESR is decreased in: polycythemia, hyperviscosity, sickle cell anemia, leukemia, low plasma protein (liver, kidney disease) and congestive heart failure.

Reference: Henry's Clinical Diagnosis and Management by Laboratory Methods, 22nd edition, page-521.

Note :- Suggested Clinical Correlation *

****End Of Report****

Dr.Gaurav Rastogi MD,Pathology
Senior Consultant

Dr.Sushmitha Reddy A
Consultant Pathologist


Dr.K Kiran
Consultant Pathologist

Dr.A.Ramya Priyadarshini
Consultant Pathologist

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD

Patient Name : **Mr.ANIL KUMAR REDDY**Age / Gender : **38 Y(s) / Male**

Phone : 8008931117

Ref.Dr. : SELF

Req.No : **JBH092500684** - 2500796541Report Status : **Preliminary**

Sample Type : Serum

Registered On : 20-09-2025 09:17

Collected On : 20-09-2025 11:13

Received On : 20-09-2025 11:19

Reported On : 20-09-2025 13:18

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DEPARTMENT OF CLINICAL BIOCHEMISTRY

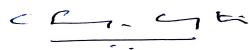
25 Hydroxy Vitamin D**25 Hydroxy Vitamin D.** : 18.37 ng/mL

Method : CLIA

Deficiency : < 20
Insufficiency: 20 - 30
Sufficiency : 30 - 100
Upper limit of safety : > 100

Note :- Suggested Clinical Correlation *

End Of Report

**Dr.Vijay Babu B,MBBS.MD**
Consultant Biochemistry**Dr.Sowmya Gayatri .C**
M.B.B.S MD
Consultant Biochemistry**Dr.S.A.Nabi Ph.D**
Sr.Biochemist

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD

Patient Name : **Mr.ANIL KUMAR REDDY**Age / Gender : **38 Y(s) / Male**

Phone : 8008931117

Ref.Dr. : SELF

Req.No : **JBH092500684** - 2500796541**Report Status : Preliminary**

Sample Type : Serum

Registered On : 20-09-2025 09:17

Collected On : 20-09-2025 11:12

Received On : 20-09-2025 11:19

Reported On : 20-09-2025 13:31

Scan to Validate



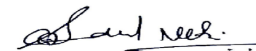
DEPARTMENT OF CLINICAL BIOCHEMISTRY

Iron with TIBC

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Iron <i>Method : TPTZ</i>	: 134.20	µg/dL	70-180
Total Iron Binding Capacity (TIBC) <i>Method : Nitroso-PSAP</i>	: 486.05	µg/dL	255-450

Reference:Beckman kit literature.

Note :- Suggested Clinical Correlation *

****End Of Report******Dr.Vijay Babu B,MBBS.MD**
Consultant Biochemistry**Dr.Sowmya Gayatri .C**
M.B.B.S MD
Consultant Biochemistry
Dr.S.A.Nabi Ph.D
Sr.Biochemist

Processing Location :

Rd #68, Jubilee Hills, Hyderabad- 500033
Email: centrallab@luciddiagnostics.com

Client : HCL AVITAS PVT LTD
Patient Name : **Mr.ANIL KUMAR REDDY**

Age / Gender : **38 Y(s) / Male**

Phone : 8008931117

Ref.Dr. : SELF

Req.No : **JBH092500684** - 2500796542

Report Status : **Preliminary**

Sample Type : Plasma - F

Registered On : 20-09-2025 09:17

Collected On : 20-09-2025 11:12

Received On : 20-09-2025 11:19

Reported On : 20-09-2025 12:36

Scan to Validate



DEPARTMENT OF CLINICAL BIOCHEMISTRY

Glucose, Fasting (FBS)

TEST NAME	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Fasting Plasma Glucose	: 94	mg/dL	Normal : 70 - 100 Prediabetes : 100-125 Diabetic : ≥ 126
<i>Method : Hexokinase</i>			

Interpretation:

Criteria for diagnosis of Diabetes mellitus.

FPG ≥ 126 mg/dL, Fasting is defined as no caloric intake for atleast 8 hours.

Remarks:

Fasting hypoglycemia may be observed in persons taking certain diabetes medications, antibiotics and alcoholic beverages. Additionally intestinal disorders, endocrine disorders, insulin surge, nature of diet, reduced food intake than usual and stress are among several other factors. Please correlate clinically.

Reference: American Diabetes association guidelines 2021.

Note :- Suggested Clinical Correlation *

****End Of Report****

[Signature]

Dr.Vijay Babu B,MBBS.MD
Consultant Biochemistry

Dr.Sowmya Gayatri .C
M.B.B.S MD
Consultant Biochemistry

Dr.S.A.Nabi Ph.D
Sr.Biochemist

Mr. Anil Kumar Reddy
JBH092500684.
Weight ÷ 72 kg
Height ÷ 165 cm
BP ÷ 122/85

MEDICAL CERTIFICATE

I, Dr. Sreenivas do hereby certify that I have carefully examined
Sri./Smt. Mr. Anil Kumar (Whose signature is given below), son / daughter
of K. Kasturini Reddy is physically fit / unfit to join school / organization / undergo
professional education.

Signature of Candidate / Guardian:

Signature of Doctor: Registration No:

Place: Hyderabad

Date: 20/9/2025



Regd. Office:

#18, Rao & Raju Colony, Road No. 2, Banjara Hills, Hyderabad - 500 034

T: +91 40 4419 4444, Toll free No.: 18001234577



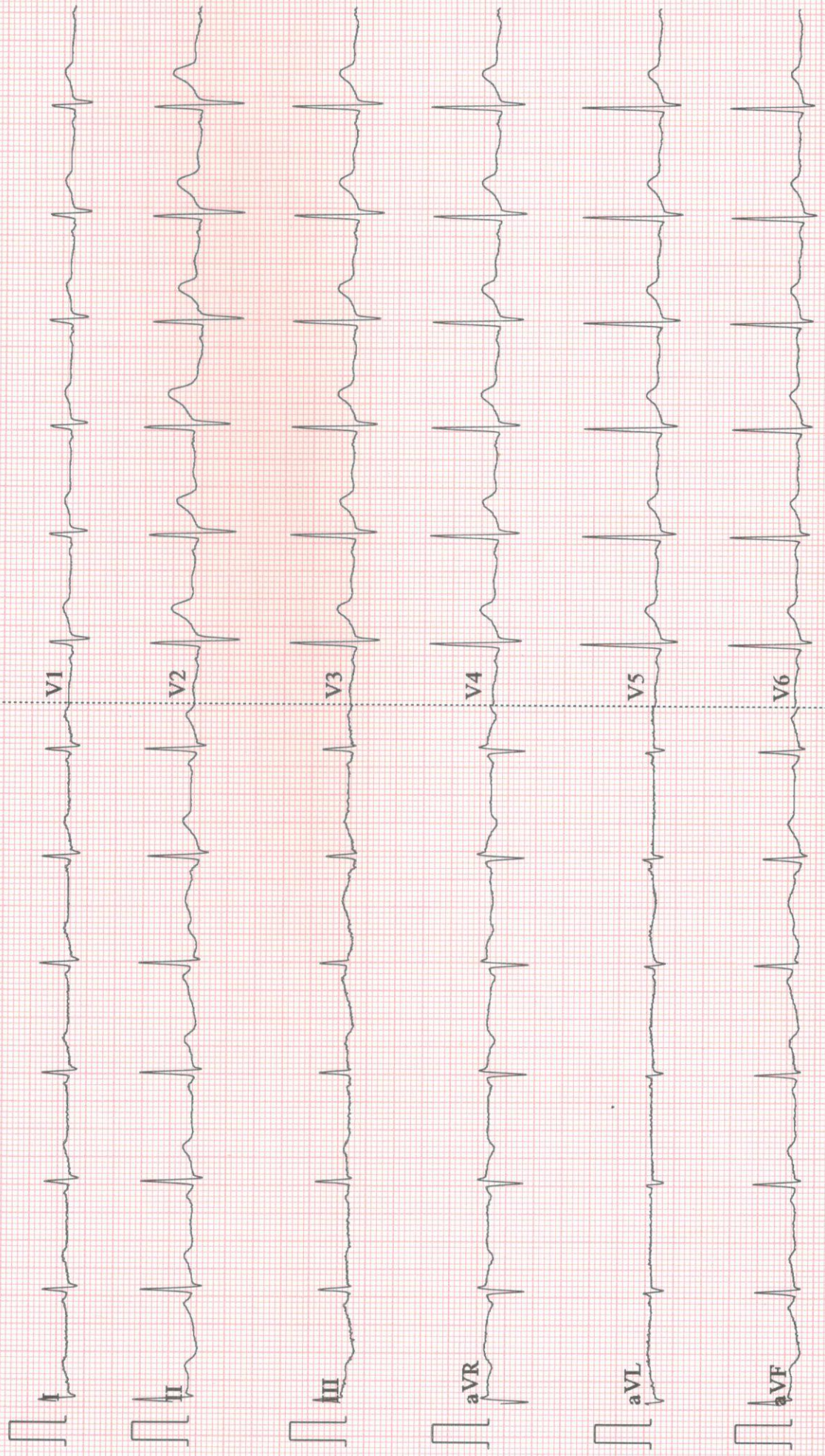
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ID: 684
MR ANIL KUMAR REDDY
Male 38Years

20-09-2025 09:52:12 AM
HR : 77 bpm
P : 106 ms
PR : 135 ms
QRS : 79 ms
QT/QTcBz : 358/407 ms
P/QRST : 57/69/58 °
RV5/SV1 : 1.387/0.288 mV

Diagnosis Information:
Sinus Rhythm
Normal ECG

Report Confirmed by:





Patient Name	: Mr . ANIL KUMAR REDDY	Reg No	: JBH092500684
Age/Gender	: 38 Y(s) / Male	Reg Date	: 20-Sep-2025 09:17
Ref.Dr	: SELF	Reported Date	: 20-Sep-2025 13:52
Reference	: HCL AVITAS PVT LTD		

ULTRASOUND WHOLE ABDOMEN

LIVER: Normal in size (13.2 cms) and echotexture with smooth contours. No focal lesions. Intrahepatic biliary and vascular radicles are normal.

PORTAL VEIN: Normal. **CBD:** Normal.

GALL BLADDER: Well distended. No evidence of calculi / wall thickening.

SPLEEN: Normal in size (8.0 cms) and echopattern.

PANCREAS: Poor window (Obscured by bowel artefacts).

AORTA & IVC: Normal in calibre. No pre / para aortic lymphadenopathy.

KIDNEYS: Normal in size, with smooth contours. Parenchymal texture normal. Cortico medullary differentiation well maintained. No calyectases / calculi.

Right Kidney measures 9.9 x 4.1 cms.

Left Kidney measures 10.7 x 4.6 cms.

URINARY BLADDER: Well distended. No evidence of wall thickening / calculi.

PROSTATE: Normal in size and echotexture.

Prostate measuring 2.6 x 4.2 x 3.6 cms, Volume - 21 cc.

No free fluid in the abdomen.

IMPRESSION:

*** No significant abnormality to the visualised extent in the present study.**

SUGGESTED CLINICAL CORRELATION AND FOLLOW UP.

Dr.Sunitha Lingareddy
Consultant Radiologist

Dr.Praveen Jagarlamudi
Consultant Radiologist

Dr.Vikas Reddy P
Consultant Radiologist

Dr.Rajya Lakshmi.D
Consultant Radiologist

Dr.Salila I
Consultant Radiologist

Dr.Guthireddy Akhila
Consultant Radiologist

Regd. Office:

Plot # 18, Rao & Raju Colony, Road No.2, Banjara Hills, Hyderabad-500 034.
Ph: +91 40 4419 4444, Toll Free No. : 1800 123 4577





Patient Name	: Mr . ANIL KUMAR REDDY	Reg No	: JBH092500684
Age/Gender	: 38 Y(s) / Male	Reg Date	: 20-Sep-2025 09:17
Ref.Dr	: SELF	Reported Date	: 20-Sep-2025 13:49
Reference	: HCL AVITAS PVT LTD		

X-RAY CHEST PA VIEW**(Rotation +)**

Soft tissues / bony cage normal.

Trachea and Mediastinal structures are normal.

Heart size and configuration are normal.

Aorta and pulmonary vascularity are normal.

Lung parenchyma and CP angles are clear.

Bilateral hilae and diaphragmatic contours are normal.

IMPRESSION:*** NORMAL STUDY.**

SUGGESTED CLINICAL CORRELATION AND FOLLOW UP.

Dr.Sunitha Lingareddy
Consultant Radiologist**Dr.Praveen Jagarlamudi**
Consultant Radiologist**Dr.Vikas Reddy P**
Consultant Radiologist**Dr.Rajya Lakshmi.D**
Consultant Radiologist**Dr.Salila I**
Consultant Radiologist**Dr.Guthireddy Akhila**
Consultant Radiologist**Regd. Office:**Plot # 18, Rao & Raju Colony, Road No.2, Banjara Hills, Hyderabad-500 034.
Ph: +91 40 4419 4444, Toll Free No. : 1800 123 4577

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